



Fiona Stanley Fremantle Hospitals Group

Policy and Procedure

Interviewing Mental Health Consumers: Managing Risk

Reference #: FSFH-MENH-PRO-0006

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital	Mental Health Service	Medical, Nursing, Allied Health, SIMS and Security Services, Pharmacy

1. Introduction

When interviewing a mental health consumer clinicians may potentially be at risk of harm as a result of unpredictable behaviours e.g., violence or aggression. This policy and procedure outlines requirements and strategies which will assist in mitigating the risk of harm/injury to clinicians should a consumer/family member/carer become aggressive or violent during an interview.

2. Terminology

Elevated risk consumer

For the purposes of this policy is a consumer who may currently have propensity for violence and or is highly aroused, disturbed or agitated and who may or may not have a history of aggression or violence.

3. Policy

- The [WA Occupational Safety and Health Act 1984](#) applies a duty of care obligation for employers to provide (as far as practical) a workplace free of hazards and the risk of harm. This includes the requirement to 'Take all practical measures to identify hazards, assess risks and to control risks'.
- To maximise the safety of consumers and staff the service will ensure that:
 - all clinical staff are compliant with Aggression Prevention Intervention training

- all clinical staff are trained how to initiate a duress response
- a duress pendant is worn by the clinician during any interview
- interviews will not be conducted in patient bedrooms
- A list of mental health service interview rooms and their suitability for consumers of varying risk levels is available in [Appendix 1](#).
- All interview rooms must have/be:
 - a clear exit path, with doors and furniture positioned to allow safe egress
 - minimal furniture (chairs, desks, filing cabinet secured to wall)
 - free of objects that can be used as weapons and/or barricade

4. Procedure

4.1. Risk assessment

- All consumers will be routinely risk assessed on admission, discharge or at significant events.
- Identified risks for aggression/violence will be communicated at each clinical handover or team meeting using the iSoBAR format and/or iSOFT handover.
- Risk will be documented in the consumer's digital medical record (DMR) in line with the FSFHG Clinical risk assessment and management policy and procedure.

4.2. Interview preparation

- The staff member conducting the interview will review the following to inform their assessment of the consumer's risk for violence/aggression:
 - most recent risk assessment in the DMR and/or Psychiatric Online Information System (PSOLIS)
 - PSOLIS alerts or receive handover regarding PSOLIS alerts
 - recent progress notes to gauge mental state and any significant changes indicating an increased level of risk
 - current mental state
- In the inpatient setting the staff member will also obtain a clinical handover from the allocated Nurse to be informed of any information relevant to the conduct of the interview.
- For patients assessed as having an elevated risk of aggression/violence consider the following additional precautions and implement those which are feasible:

- identify an appropriate interview room, consider going outside or to a less intimidating environment
- invite the patient to the interview explaining the purpose of the interview using a trauma informed approach and appropriate de-escalation skills throughout
- use the agitation and arousal chart to offer medication if all other non-pharmacological de-escalation attempts are exhausted
- inform the Shift Coordinator/Nurse Unit Manager/Treatment Team of the commencement of the interview
- arrange for the patient's personal support person and peer experience worker/coordinator to attend
- engage Aboriginal Mental Health Liaison for cultural safety for Aboriginal and/or Torres Strait Islander patients
- arrange to have additional staff members attend who are appropriately skilled and trained
- arrange assistance from Security/SIMS staff

4.3. Engaging Security/SIMS staff

- Occasions where Security/SIMS staff should be utilised during an interview include but are not limited to:
 - highly aroused, aggressive, unknown or unpredictable behaviours from consumers/or family/carer
 - consumers/family/carer with a known history of aggressive or assaultive behaviour
- When a decision is made to request Security/SIMS presence the Shift Coordinator/interviewer will:
 - contact Security/SIMS to request their attendance and appropriately brief Security/SIMS on their expected role
 - simultaneously consider engaging with culturally appropriate supports
 - provide a further update on arrival
 - ensure the interview does not commence until Security/SIMS staff are present.

4.4. During the interview

- The staff member/s will ensure that:
 - the consumer is seated furthest away from the exit
 - staff are located closest to the exit

- there is adequate spacing between the clinician and the consumer
- staff have a duress pendant/alarm readily available
- the consumer is observed for signs of distress or increased levels of agitation and arousal
- tactful, trauma informed, non-confrontational intervention/de-escalation techniques are used to defuse a situation if required
- a duress/code black response is activated if feeling threatened and remove themselves to a staff only safety zone

4.5. Post elevated risk interview

- Staff will inform the Shift Coordinator/NUM/Treatment Team of the completion of any elevated risk interview and the outcome.
- The outcome of the interview and any changes to the consumer's risk assessment and/or overall treatment plan will be documented in the patient's DMR and Treatment Support Discharge Plan and discussed with the multidisciplinary team
- In the event of any actual/threatened aggressive/violent incident during an interview the staff members involved will:
 - report any incidents or near misses regardless of injury to their manager
 - complete Combined Hazard or Incident Reporting (CHoIR)
 - complete Datix Clinical Incident Management System specifying a notifiable incident to the Chief Psychiatrist
 - add a new PSOLIS alert if clinically indicated
- Senior staff will:
 - ensure any person affected by the incident is safe and that the necessary steps are taken to support and treat persons involved
 - make the surroundings safe to prevent immediate recurrence of the incident
 - arrange appropriate debrief for all staff involved in the incident, including Security/SIMS staff
 - inform staff of the availability of the Employee Assistance Program

5. Compliance/performance monitoring

Incidents of aggression/violence will be monitored:

- monthly by the Service 5 Safety Quality Risk Committee

- quarterly by the Service 5 WHS Committees

6. Related policy documents

WA Health:

- [Missing person MP-0012/16](#)
- [Clinical Incident Management Policy 2019](#) MP 0122/19
- [Policy for Mandatory Reporting of notifiable incidents to the Chief Psychiatrist](#)

SMHS:

- [Critical incident stress management](#)
- [Employee assistance program](#)
- [Workplace aggression and violence](#)
- [WHS Hazard and Incident Management](#)
- [Safety skills training](#)

FSFHG

- Clinical risk assessment and management (FSFH-HW-POL-0017)
- Anticipating and reducing the risk of unpredictable behaviours (FSFH-MENH-POL-0027)
- Restrictive practices (FSFH-MENH-POL-0019)
- Clinical Handover (FSFH-HW-POL-0035)
- Clinical Documentation (FSFH-HW-POL-0039)
- Code Black Mental Health Unit (FSFH-MNH-POL-0014)
- FSH-Code Black
- Code Black FH
- Duress Alarms and Response (FSFH-MENH-PRO-0005)

7. Related documents

- [Occupational Safety and Health Act 1984](#)

8. Related standards

NSQHS standards:

- Comprehensive Care
- Communicating for Safety
- Recognising and Responding to Acute Deterioration

Chief Psychiatrist Standards for Clinical Care:

- Care Planning
- Risk Assessment and Management

9. Authorisation

EXECUTIVE SPONSOR: Service Director, Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	10/2016	CNS, CI	FHMHS CGC	Policy Committee	10/2019
2	12/2020	CNS, CI	Service 5 SQRC	FSFHG Policy Committee	12/2023
3	01/2024	CNS, CI		Service 5 Safety Quality and Risk Committee	07/2027

10. Appendices

10.1. Appendix 1: Interview Rooms

Location	Interview Room	Aggression/violence risk
Ward W4.1	W4.59	Elevated risk
	W4.76	Elevated risk
Ward W4.2	Patient lounge	Elevated risk
	W4.122	
Ward W4.3	Group room W4.238	Elevated risk
Ward W5.1	W5.108	
	W5.93	
Triage	W4.25	Elevated risk
	W4.30	Elevated risk
FH Outpatients	W5.42b	Elevated risk
	W5.43	Elevated risk
	Consulting rooms/Offices	
	Other interview rooms	
FOAMHS	Interview Room L5-52	
	Medical staff offices L5-59 to 62	
Youth Unit	Family room D-GL0012	
	Comfort Room D-GL0131	
	Interview room D- GL0132	
MBU	Interview room D-GL0074	
	Family meeting room D-GL0073	
	Activity room D-GL0080	

Location	Interview Room	Aggression/violence risk
MHAU	Interview Room 4 D-GL0036	Elevated risk
	Interview Room 5 D-GL0036	Elevated risk
YCATT	SRS as per room roster	
	MHAU interview room	Elevated risk
APTS	APTS Clinic Room SRS Level 2	
	APTS Clinic Room Day Medical Procedures Unit Level 1.	
	FH In reach Clinic W5.25	
YRS	Moora	
	Diena Koori Bidi	
Kara Maar	Room 1	
	Room 2	Elevated risk
	Room 3	
	Room 4	Elevated risk